

Carbohydrates Metabolism: Galactose and Fructose Metabolism

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Objectives:

- At the end of this chapter, it is expected to;
 - Describe the metabolism of fructose, galactose, and other dietary carbohydrates
 - Explain how fructose, galactose, and other dietary carbohydrates feed in to glycolytic pathway
 - Explain the metabolic conditions due to defect in fructose, galactose, and other dietary carbohydrates metabolism
 - Describe the reaction, importance and consequences of polyol pathway

Fructose, galactose, Lactose,
Sucrose, Glycogen and starch
feed into the glycolytic pathway

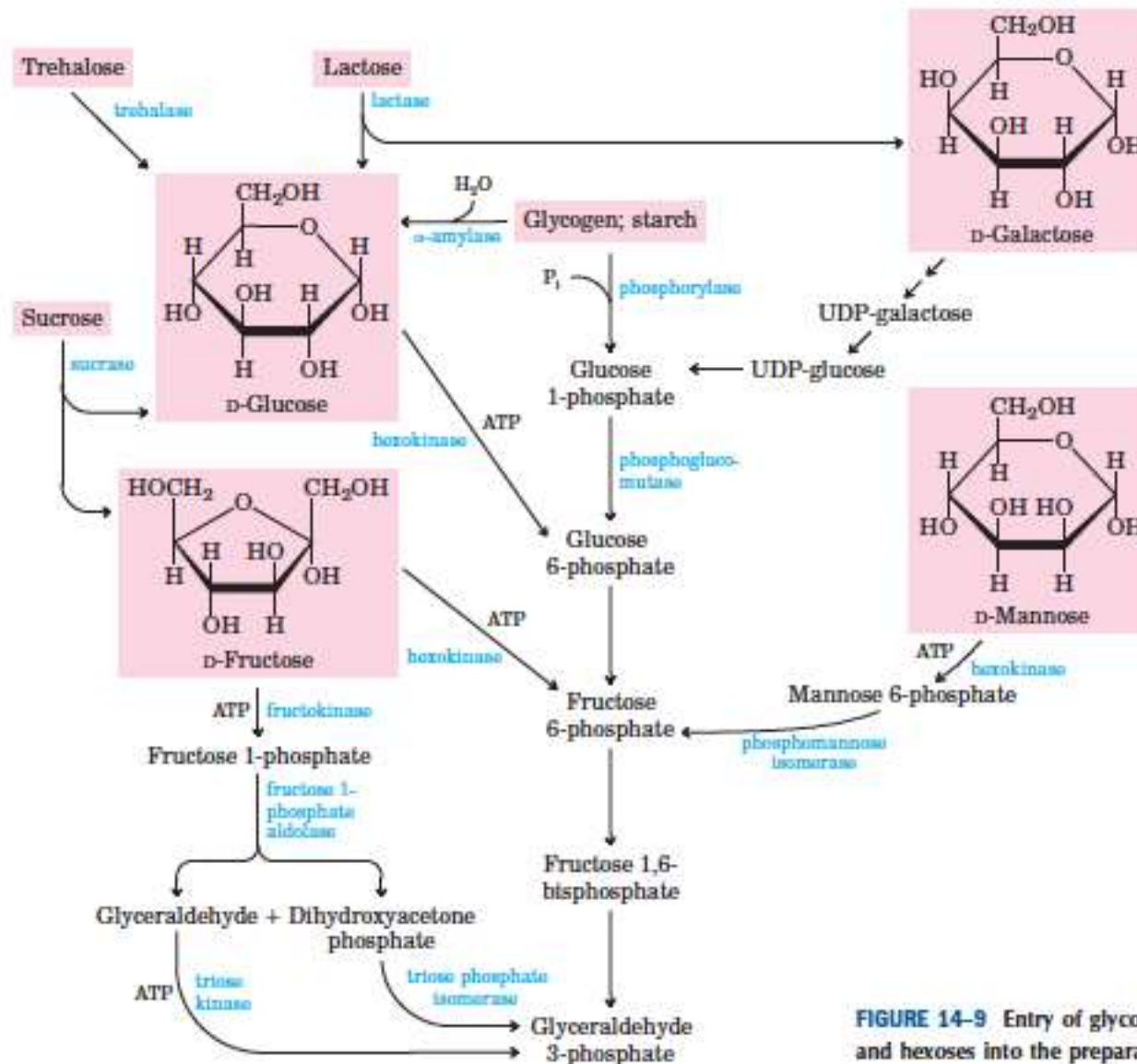
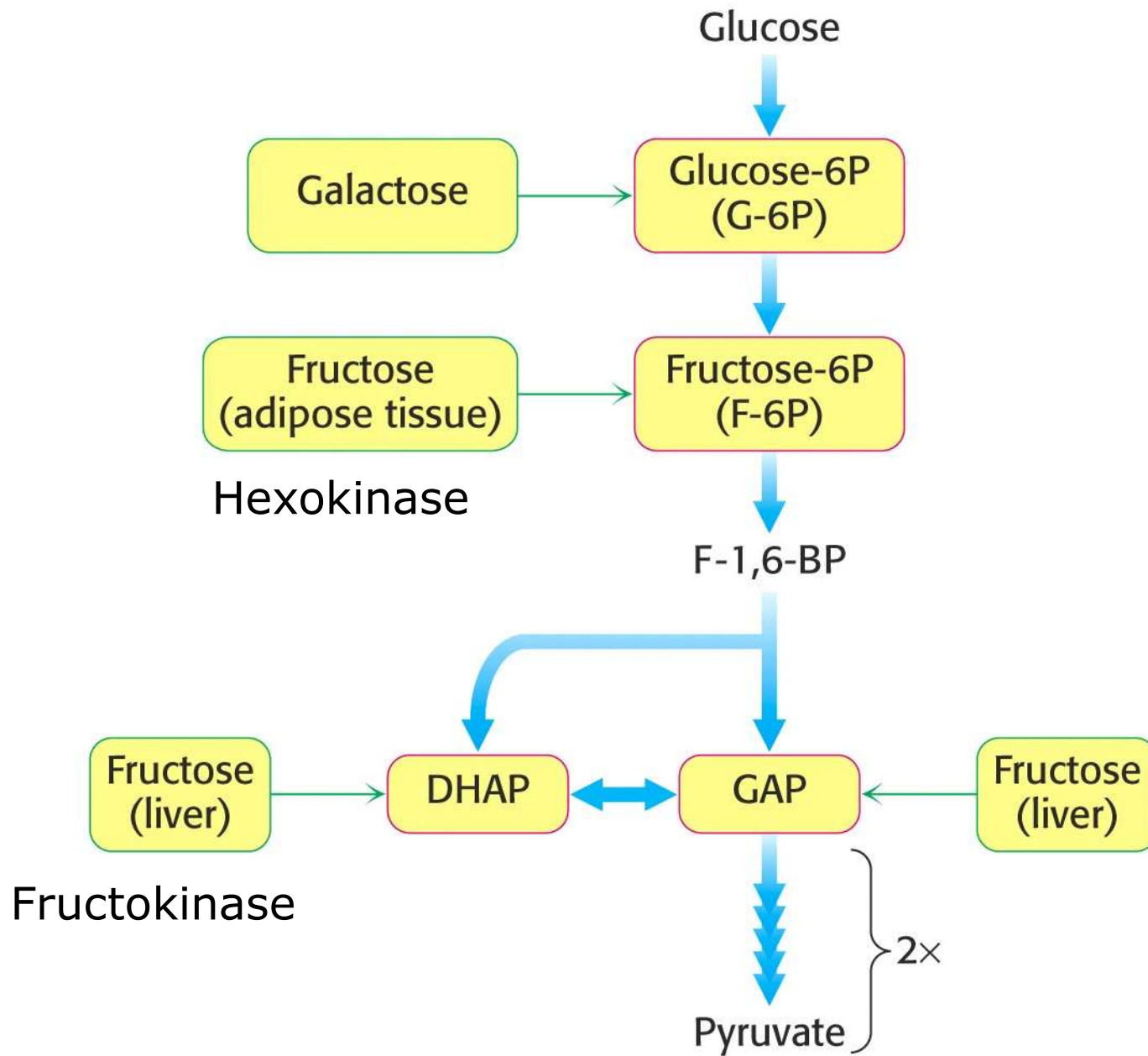
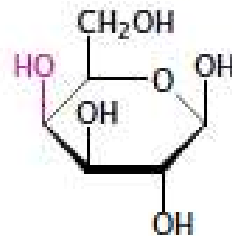


FIGURE 14-9 Entry of glycogen, starch, disaccharides, and hexoses into the preparatory stage of glycolysis.

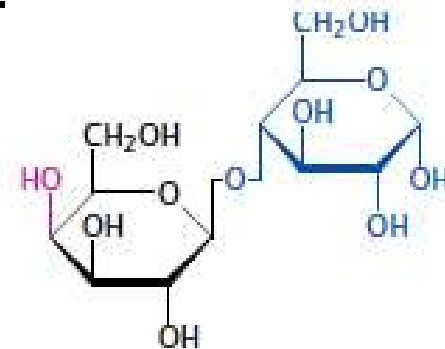


Lactose and galactose

- Lactose - a disaccharide of glucose and galactose
- The major carbohydrate contained in milk.
- Cleaved at the brush border of the small intestine;
 - And the monosaccharide fragments are absorbed and passed along to the liver.
 - The enzyme that accomplishes the cleavage is lactase or, more precisely, β -galactosidase.



β -D-galactose

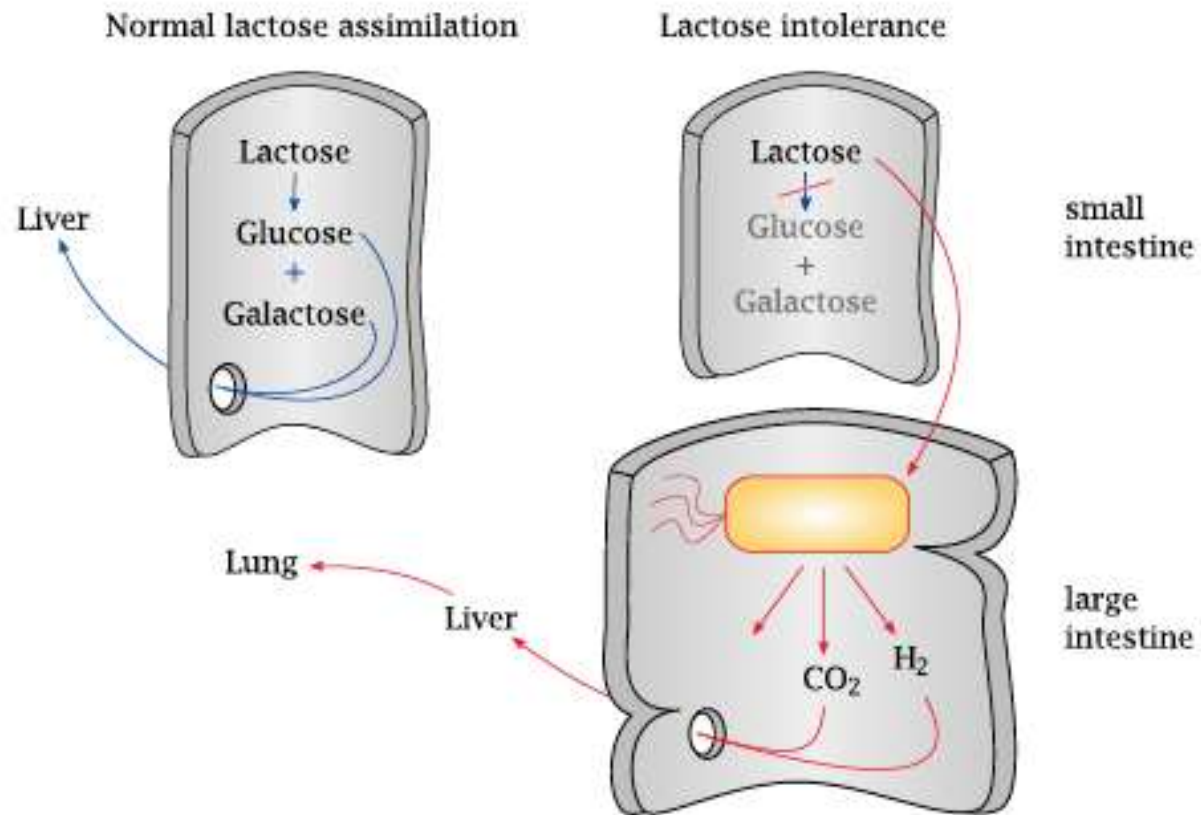


β -D-galactosyl-(1 $\rightarrow$ 4)-D-glucoside (lactose)

Lactose intolerance

- A deficiency of the lactase enzyme in the small intestine
- Found frequently in people of East Asian descent who are past their infant age
- Lactose not cleaved in small intestine – moved down to large intestine where get metabolized by bacteria's (E.Coli).
- Mixed acid fermentation → Formic acid (HCOOH), which is then cleaved by formic acid lyase to H_2 and CO_2
- The aberrant fermentation and gas formation leads to abdominal discomfort and diarrhea

Lactose intolerance



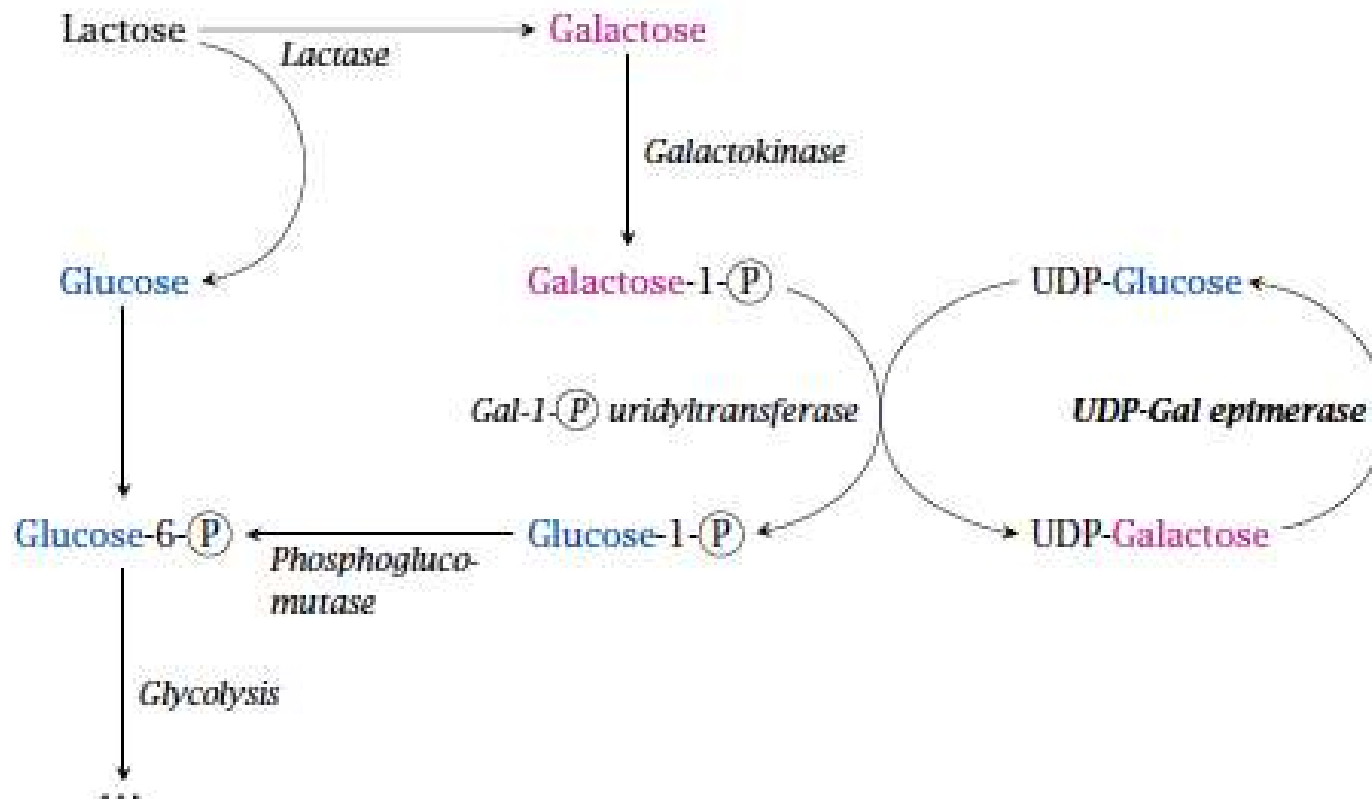
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A case file: Lactose intolerance

- A patient with lactose intolerance presents with symptoms of bloating, gas, diarrhea, and abdominal pain after consuming dairy products.
- Biochemical changes:
 - Deficiency of the enzyme lactase.
 - Production of gas (hydrogen, methane).
 - Osmotic effect draws water into the colon, causing diarrhea.
- Diagnosis:
 - A lactose tolerance test
 - Genetic testing.
- Management:
 - Avoiding lactose-containing foods,
 - Using lactase supplements, or
 - Consuming lactose-free dairy products to alleviate symptoms and maintain a balanced diet.

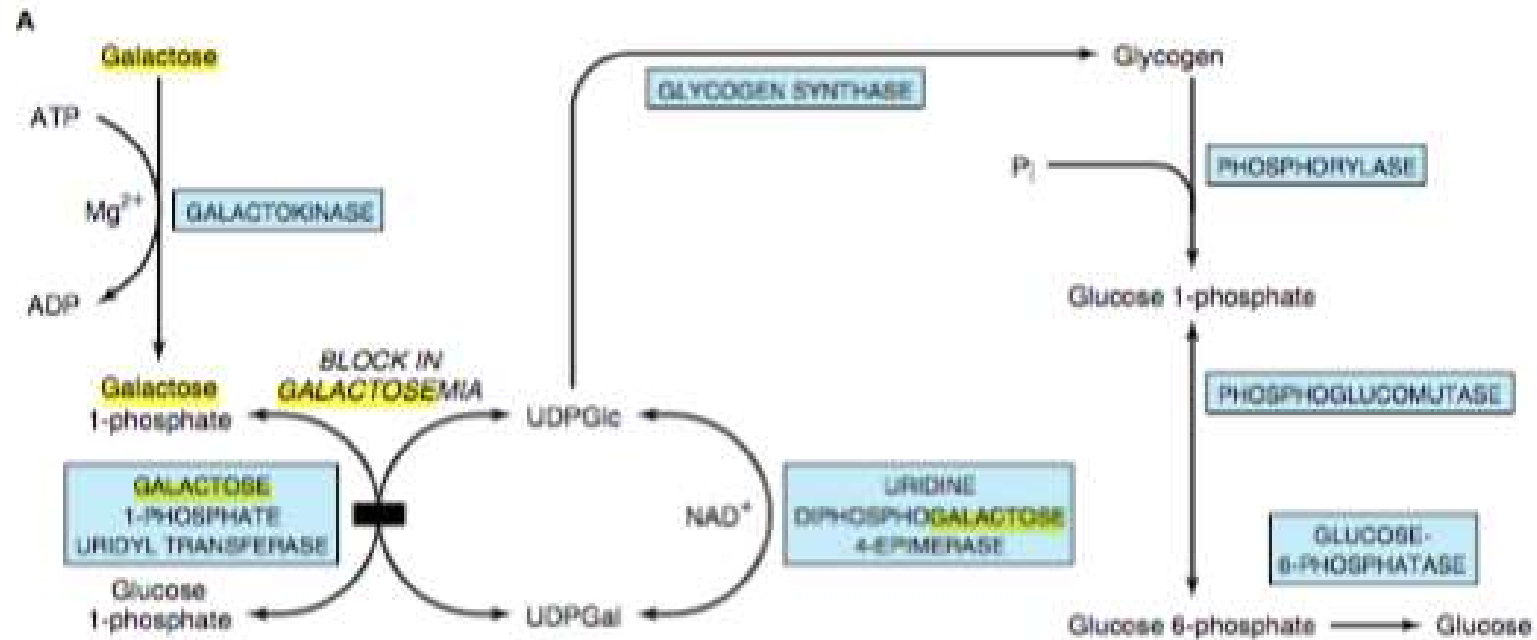
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The Leloir pathway: Galactose is utilized by conversion to glucose

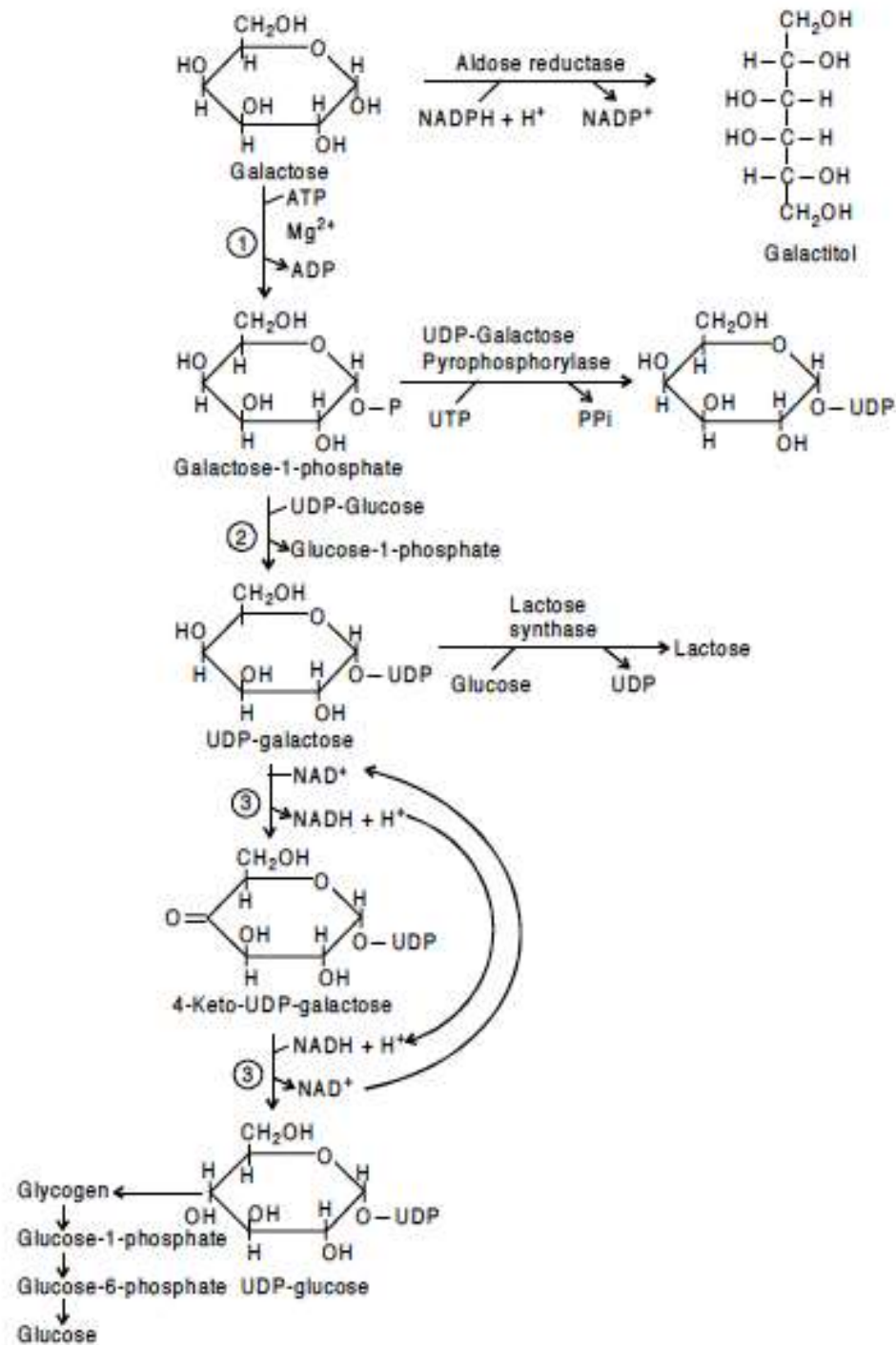


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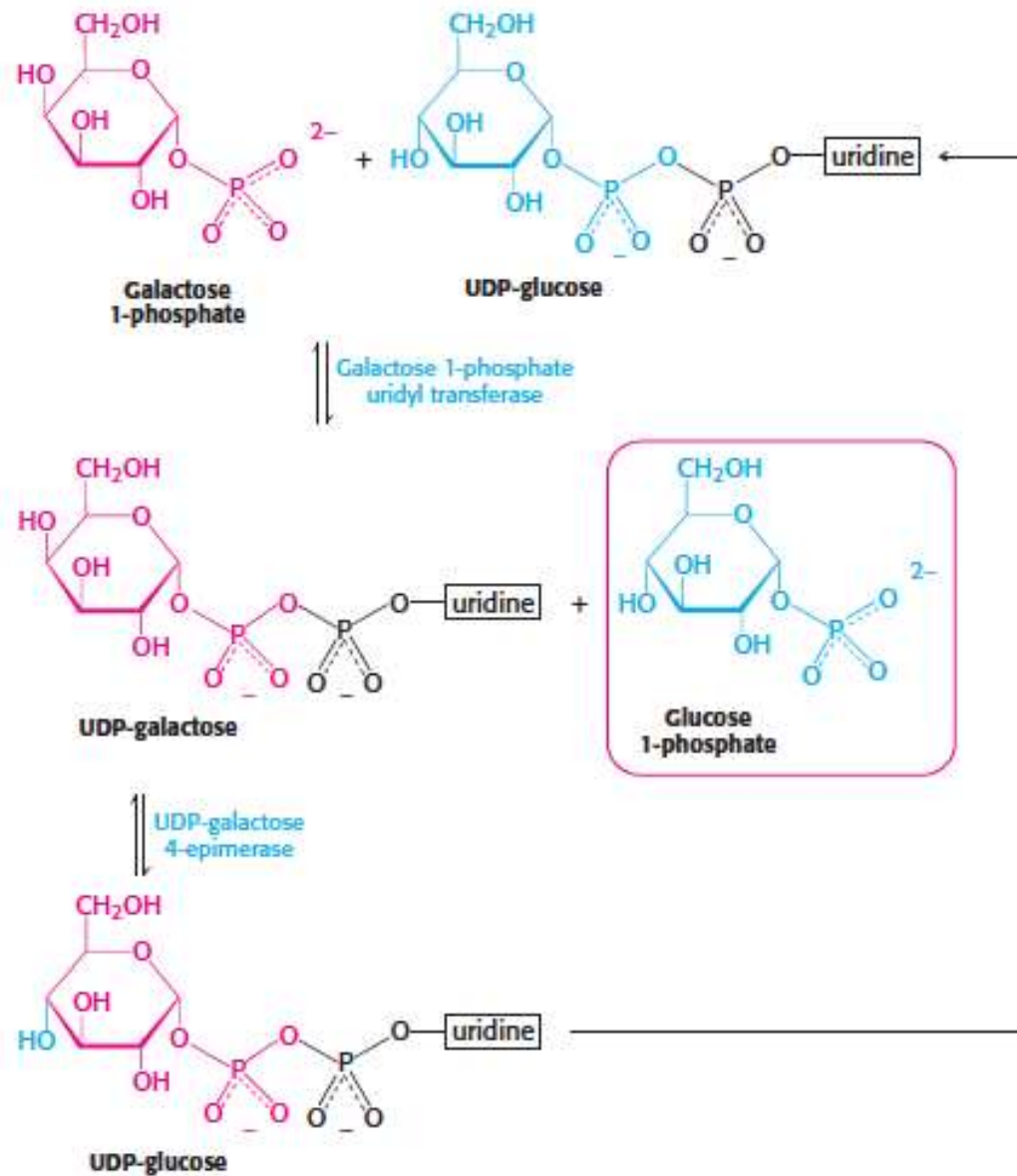
Galactose is converted to glucose-6P via a four step reaction involving UDP-glucose (The Leloir pathway)



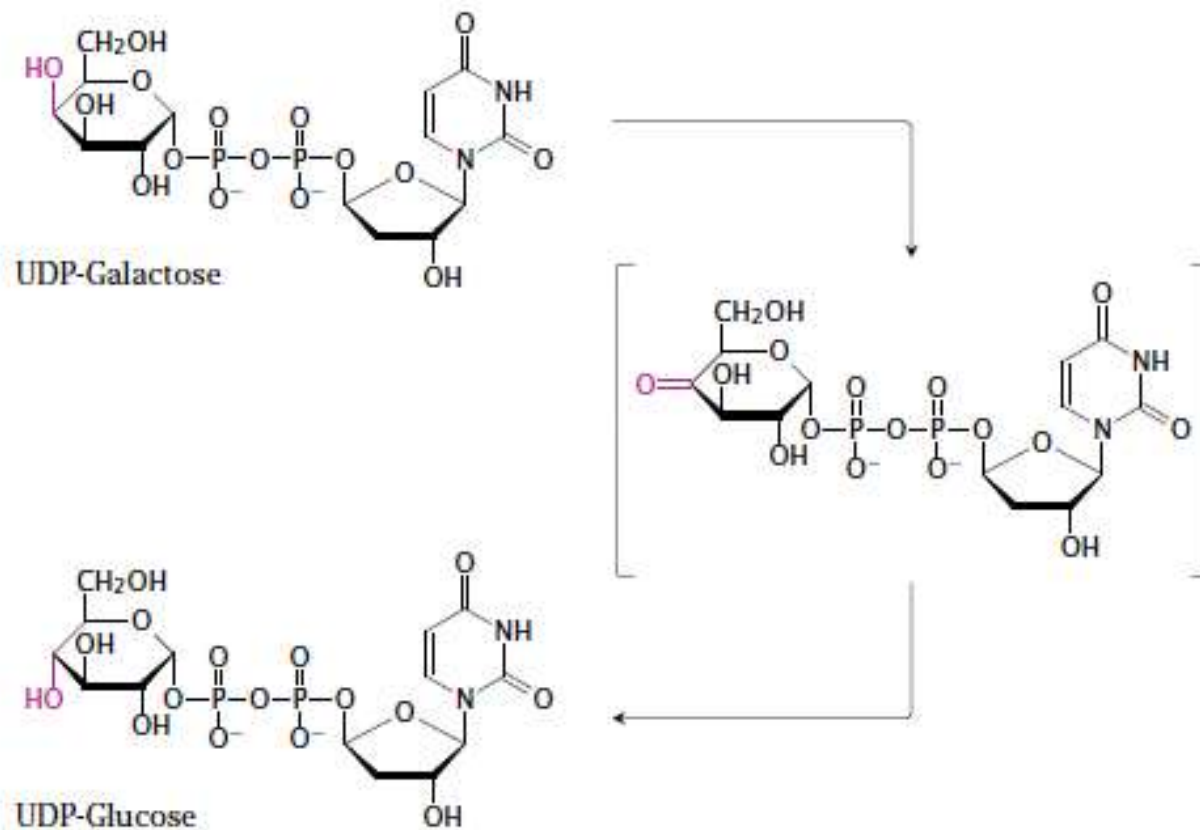
The Leloir pathway:



Galactose is utilized by conversion to glucose



Mechanism of UDP-galactose epimerase



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Galactosemia

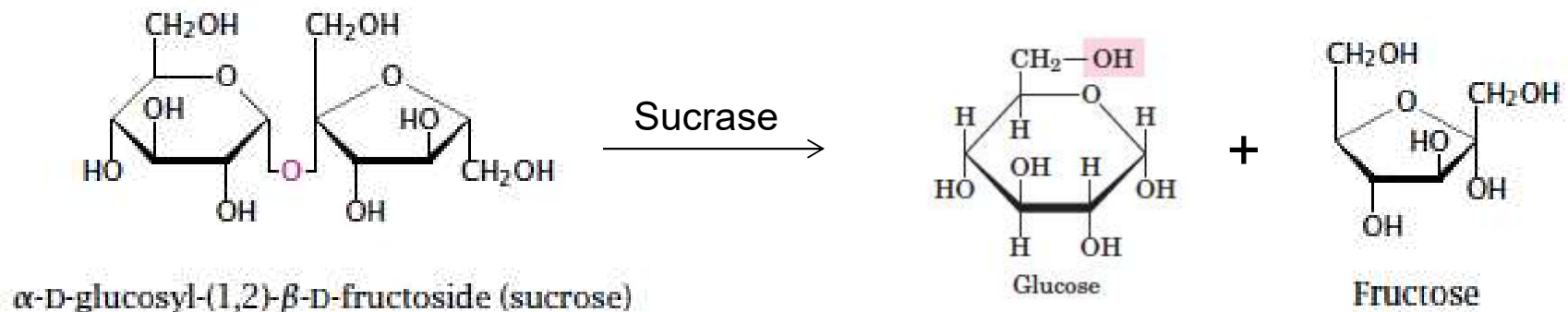
- means “galactose in the blood.”

Type	Enzyme deficiency	Accumulating metabolites
I	galactose-1-phosphate-uridyltransferase	galactose, galactose-1-phosphate, galactitol, galactonate
II	galactokinase	galactose, galactitol
III	UDP-galactose epimerase	galactose-1 phosphate, UDP-galactose

A case file: Galactosemia

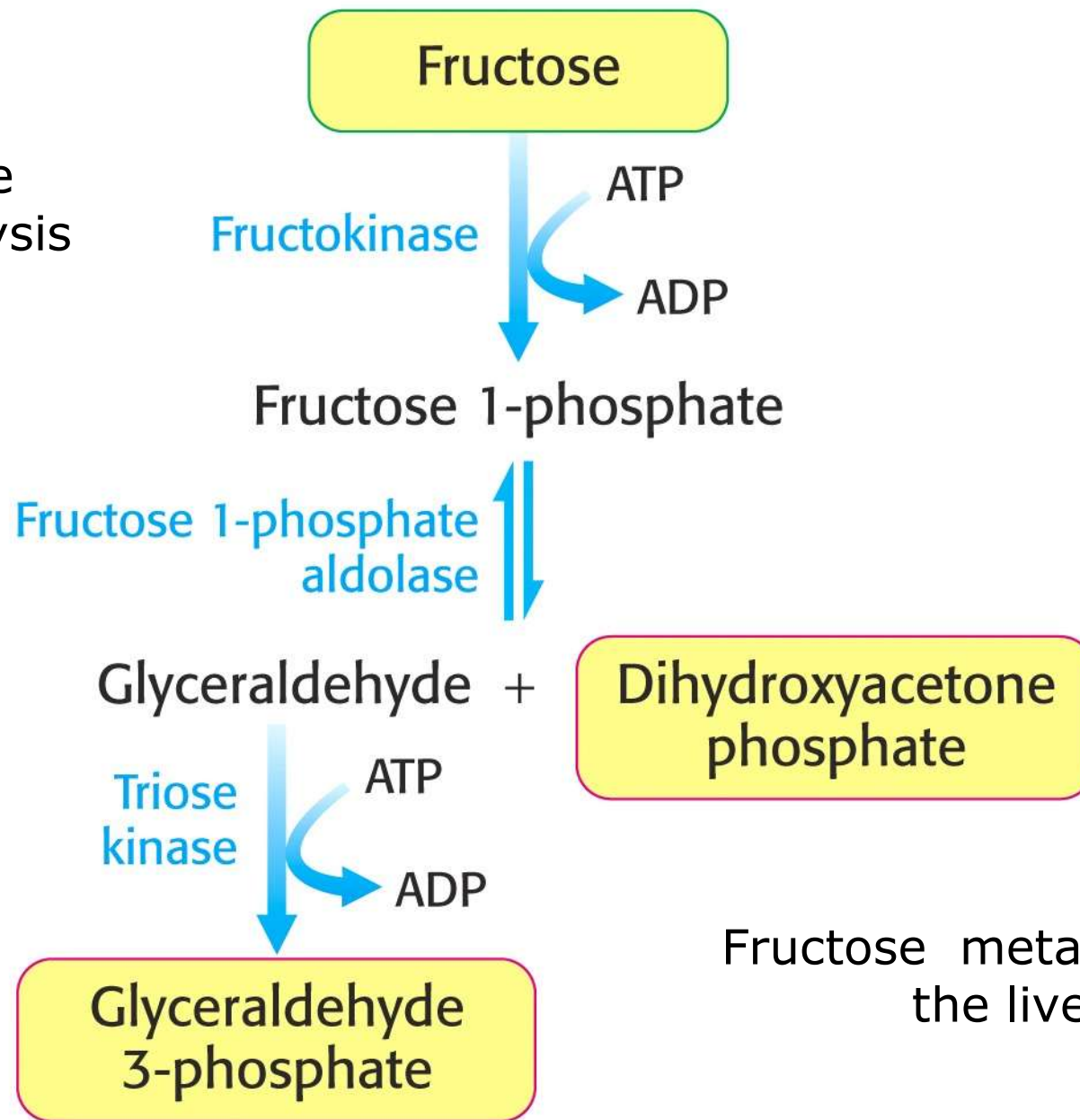
- A patient with galactosemia presents with symptoms of jaundice, vomiting, and lethargy after consuming milk or dairy products.
- Biochemical changes:
 - Deficiency of the enzyme galactose-1-phosphate uridylyltransferase (GALT).
 - Accumulation of Galactose-1-phosphate, galactitol and galactonate.
- Clinical manifestations:
 - Damage to the liver, kidneys, and nervous system, leading to symptoms such as jaundice, vomiting, lethargy, and developmental delays.
- Diagnosis:
 - Newborn screening or
 - Genetic testing for GALT mutations.
- Management:
 - Avoiding milk and dairy products,
 - Using galactose-free formula, and
 - Monitoring for complications such as cataracts, developmental delays and learning disabilities.

Degradation of sucrose



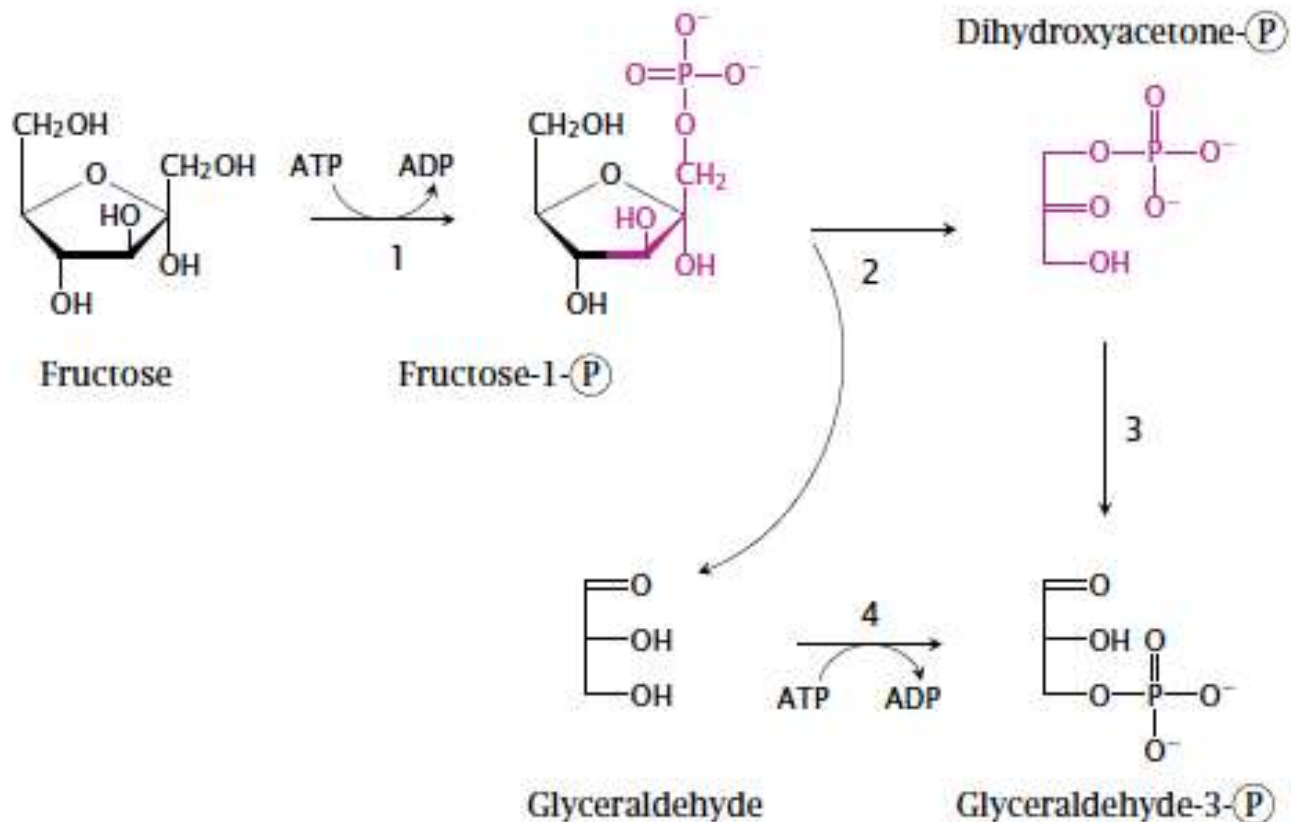
- Sucrose consists of glucose and fructose joined by a β -glycosidic bond between the carbon 1 of glucose and carbon 2 of fructose.
- The hydrolytic cleavage of sucrose occurs at the surface of the intestinal epithelial cells using β –fructosidase (sucrase).
- Both sugars are then taken up by specific transport: Glucose by the SGLT1 transporter, and fructose by the GLUT5 transporter.

In the liver,
when fructose
enters glycolysis
the PFK
reaction is
bypassed.



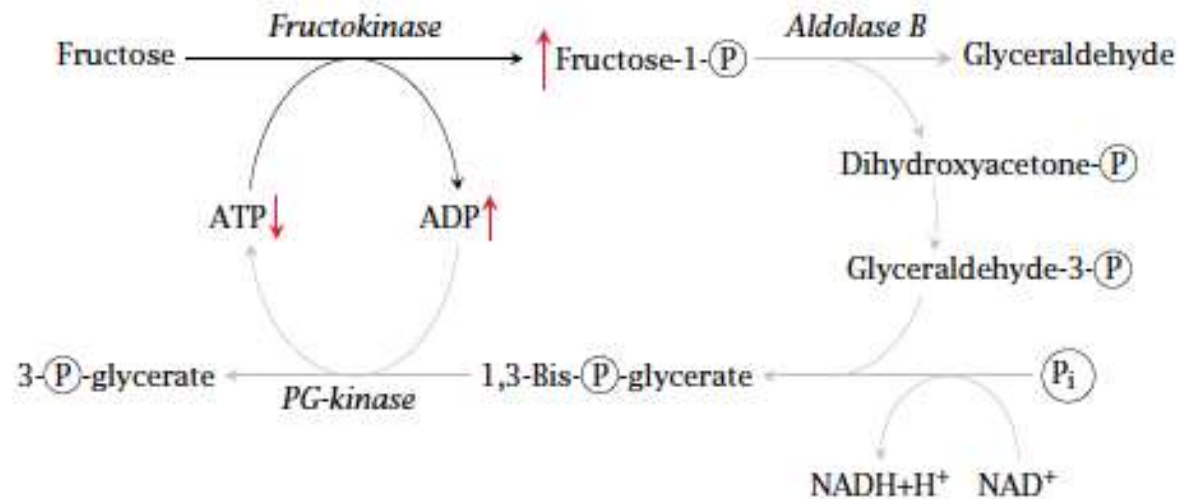
Fructose metabolism in
the liver

The fructolysis pathway



1=fructokinase; 2=aldolase B; 3=triose phosphate isomerase; 4=glyceraldehyde kinase

Fructose intolerance & Fructosemia



Fructose intolerance:

- a hereditary disease caused by a homozygous defect in the aldolase B gene.
- resulting in fructose-1-phosphate accumulation and lacking of ATP
- sooner or later damage or even destroy the cell and characterized by potentially severe liver failure.
- Large intravenous dosages of fructose can significantly deplete liver ATP

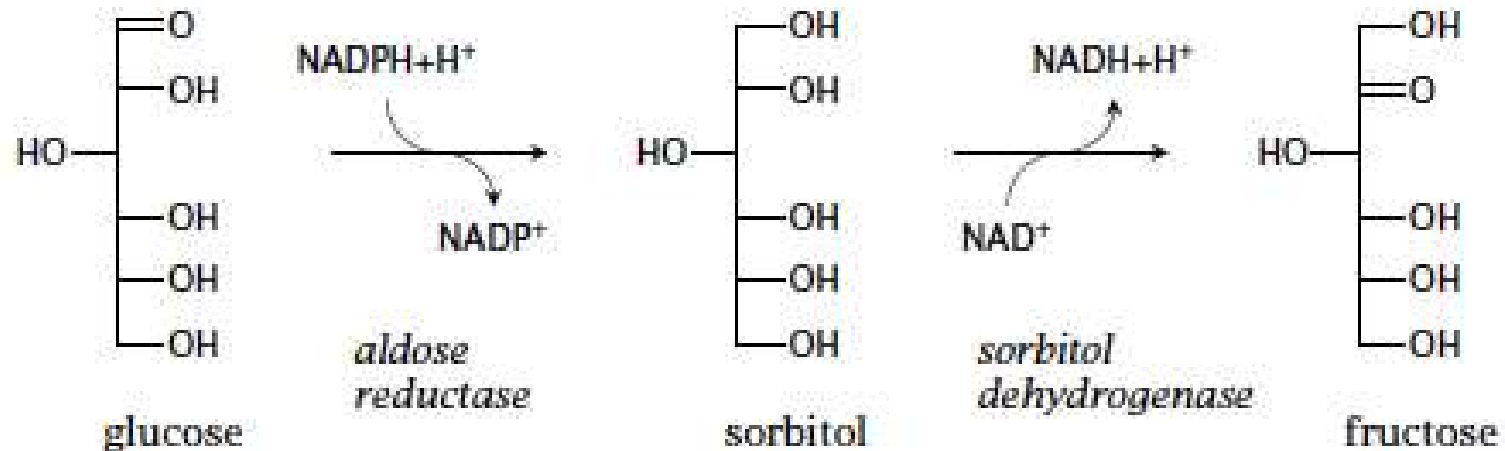
Fructosemia:

- A defect in the gene encoding fructokinase leads to a condition named fructosemia or fructosuria.
- fructose levels are increased both in the blood and the urine.
- Since fructose is not phosphorylated, no phosphate depletion occurs, and the liver cells do not incur any damage.

A case file: Fructose intolerance

- A patient with fructose intolerance presents with symptoms of abdominal pain, diarrhea, and bloating after consuming foods high in fructose, such as fruits, juices, and sweets.
- Biochemical changes:
 - Deficiency of the enzyme aldolase B.
 - Fructose-1-phosphate accumulates in the body, leading to the formation of harmful metabolites such as fructose-1,6-bisphosphate and sorbitol.
- Clinical manifestations:
 - Damage to the liver, leading to symptoms such as abdominal pain, diarrhea, and bloating.
- Diagnosis:
 - Genetic testing for aldolase B mutations or
 - A fructose tolerance test.
- Management:
 - Avoiding foods high in fructose,
 - Using low-fructose diets, and
 - Monitoring for complications such as liver damage and malnutrition.

The polyol pathway



- Aldose reductase also converts galactose to galactitol
- Conversion of glucose to fructose via the polyol pathway occurs in the seminal vesicles (male sexual organs) and fructose is found in the sperm fluid.
- Accumulation of either sorbitol or galactitol causes cataract by cell damage through swelling.

Read more...



THANK YOU...